

Solano County Health & Social Services Department

Mental Health Services
Public Health Services
Substance Abuse Services
Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Ann Edwards, Director

Aaron E. Bair, MD, MSc
EMS Agency Medical Director

EMERGENCY SERVICES BUREAU
275 Beck Avenue MS 5-240
Fairfield, Ca. 94533
(707) 784-8155 FAX (707) 421-6682
www.solanocounty.com

Jessica Tello-Evans, RN, PHN, MPH
EMS Agency Administrator

POLICY MEMORANDUM 7300

Date: May 21, 2014

REVIEWED/APPROVED BY:

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AARON E. BAIR, MD, MS, EMS AGENCY MEDICAL DIRECTOR

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JESSICA TELLO-EVANS, RN, PHN, MPH, EMS ADMINISTRATOR

**SUBJECT: EMERGENCY DEPARTMENT APPROVED FOR PEDIATRICS (EDAP) –
SPECIALTY CENTER DESIGNATION**

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, Chapter 2, Section 1797.67; Chapter 4, Article 1, Section 1797.222; Chapter 4, Article 3, Section 1798.170 and 1798.172.

PURPOSE/POLICY:

- A. Hospitals in Solano County are encouraged to utilize these guidelines for developing, evaluating, and implementing appropriate staffing and equipment for the care of children/minors in their respective emergency departments.
- B. To ensure 9-1-1 pediatric patients are transported to the most appropriate facility that is staffed, equipped, and prepared to administer emergency care to the needs of the pediatric patient.
- C. To establish the Quality Improvement (QI) requirements for the hospitals and Advanced Life Services (ALS) Ambulance Service Provider components of the Emergency Medical Services (EMS) System.

Note: See Attachment C for Solano County EDAP Introduction.

A. DEFINITIONS:

Children/Minors With Special Health Care Needs: Children/minors who have any type of condition that may affect normal growth and development. This may include physical disability, developmental or learning disability, technologic dependency, and/or chronic illness. The developmental age of children/minors with special health care needs may be younger than the chronological age. The developmental age is used when working with this population.

Emergency Department Approved for Pediatrics (EDAP): A licensed basic Emergency Department (ED) that is approved by the Solano Emergency Medical Services Cooperative (SEMSC) to receive pediatric patients from the 9-1-1 system. These EDs provide care to pediatric patients by meeting specific requirements for professional staff, quality improvement, education, support services, equipment, supplies, medications, and established policies, procedures, and protocols.

Medical Pediatric Critical Care Center (MPCCC): A licensed acute care hospital that is approved by a local EMS agency to receive critically ill non-trauma pediatric patients from the 9-1-1 system.

Pediatric Critical Care Center (PCCC): A licensed acute care hospital that is approved by a local EMS agency to receive patients from the 9-1-1 system. In addition, this center provides tertiary-level pediatric care services and serves as a referral center for critically ill and injured pediatric patients.

Pediatric Patient: Any person less than 15 years of age.

Primary Care Provider (PCP): A primary care physician is a generalist physician who provides definitive care to the undifferentiated patient at the point of first contact and takes continuing responsibility for providing the patient's care.

PTC: Pediatric Trauma Center

STEMI: ST segment Elevation Myocardial Infarction

Note: See Attachment D for a complete list of definitions.

**II. EDAP STANDARDS:
ADMINISTRATION/COORDINATION**

- A. EDAP Medical Director
1. Qualifications:
 - a. Qualified specialist in Emergency Medicine* or Pediatric Emergency Medicine
 - b. Completion of four hours of Continuing Medical Education (CME) in topics related to pediatrics annually
 - c. Current Advanced Pediatric Life Support (APLS) or Pediatric Advanced Life Support (PALS) provider
 2. Responsibilities:
 - a. Oversight of EDAP Process Improvement (PI) program
 - b. Member of hospital ED and pediatric committee
 - c. Liaison with PCCC, trauma centers, base hospitals, community hospitals, prehospital care providers, and the EMS Agency
 - d. Identify needs and facilitate pediatric education for ED physicians
 - e. Ensure pediatric disaster preparedness for the ED
 - f. Review, approve, and assist in the development of all pediatric policies and procedures

***Note:** Incumbent must have special interest, knowledge, and skill in emergency medical care of children/minors, as demonstrated by training, clinical experience, or focused CME.

- B. Designated Pediatric Consultant
1. Qualifications:
 - a. Board Certified in Pediatrics
 2. Responsibilities:
 - a. Member of hospital ED committee and pediatric committee
 - b. Participation with EDAP staff in developing and monitoring pediatric PI program, protocols, policies, and procedures
 - c. Participate in facilitation of pediatric education for ED physicians
 - d. Participation in pediatric disaster preparedness for the ED
 - e. Consult with EDAP Medical Director and Pediatric Liaison Nurse when needed
- C. Pediatric Liaison Nurse (PdLN)
1. Qualifications:
 - a. At least two years experience in pediatrics, or in an emergency department that sees pediatric patients
 - b. Experience with PI programs is recommended
 - c. Completion of a two day pediatric emergency nursing course or Emergency Nursing Pediatric Course (ENPC) provider/instructor course

- d. Current PALS or APLS provider/instructor or ENPC
- e. Completion of four hours of Board of Registered Nursing (BRN) approved Continuing Education Units (CEU) in pediatric topics annually

Note: Certified Pediatric Emergency Nurse (CPEN) Certification is recommended.

- 2. Responsibilities:
 - a. Attend Solano County EMS Agency meetings as needed in regard to pediatric care
 - b. Participate in the development and maintenance of a pediatric PI program
 - c. Collaborate with the EDAP Medical Director to ensure the ED is prepared to care for children/minors of all ages, including children/minors with special health care needs or mental illness.
 - d. Promote patient and family education in illness and injury prevention
 - e. Facilitation of ED nursing CEUs and competency evaluations for pediatrics
 - f. Liaison with PCCCs, trauma centers, base hospitals, community hospitals, prehospital care providers, and the EMS Agency
 - g. Member of selected hospital based ED and/or pediatric committees
 - h. Ensure emergency nursing preparedness for pediatric disasters
 - i. Notify the EMS Agency in writing of any change in status of EDAP Medical Director, Pediatric Consultant, and Pediatric Liaison Nurse

Note: A two day pediatric emergency nursing course should include, but not be limited to, a broad spectrum of topics including: injury prevention, resuscitation, surgical emergencies, Apparent Life Threatening Events (ALTE), death of a child/minor (to include Sudden Infant Death Syndrome [SIDS]), trauma, medical conditions, submersions, respiratory emergencies, airway management, ingestion, child/minor abuse and neglect, fever (to include bacterial and viral infections), seizures, and neonatal emergencies.

III. EDAP PERSONNEL

A. Physicians - Qualifications/Education

1. ED coverage shall be provided or directly supervised by physicians functioning as emergency physicians or pediatricians experienced in emergency care on a full-time basis (96 hours or more per month in an emergency department) 24 hours a day, seven days a week, and 365 days per year. This includes senior residents practicing at their respective hospitals only.
2. At least 75% of ED coverage shall be provided by physicians who are Board certified or demonstrate active progression in the certifying process towards emergency medicine or pediatrics.
3. Those ED physicians who are not board certified or board prepared shall be current APLS or PALS provider.
4. Complete pediatric competency evaluations annually that are age specific and include neonates, infants, children/minors, adolescents, and children/minors with special healthcare needs. Competencies may include, but not be limited to, the following: Airway management, burn care, critical care monitoring, medication delivery, device/equipment safety, pain assessment and treatment, trauma care, and vascular access.

B. Nurses - Qualifications/Education

1. At least 75% of the total Registered Nurse (RN) staff in the ED shall be a current PALS or APLS provider or instructor.
2. At least one RN per shift in the ED shall be current ENPC or have completed a two-day pediatric emergency nursing course (within the last four years).
3. Complete pediatric competency evaluations annually that are age specific and include neonates, infants, children/minors, and adolescents.
4. All nurses assigned to the ED shall attend, at a minimum; four hours of pediatric-related education approved by the Board of Registered Nursing (BRN) or by demonstrating written participation in an internal education process conducted by the EDAP program every year.
5. Complete pediatric competency evaluations annually that are age specific and include neonates, infants, children/minors, adolescents, and children/minors with special healthcare needs*. Competencies may include, but are not limited to, the following: airway management, burn care, critical care monitoring, medication delivery, device/equipment safety, pain assessment and treatment, trauma care, and vascular access.

***Note:** It is highly recommended that all nurses regularly assigned to the ED meet the above requirements

- C. Pediatric Physician/Specialty Services
 - 1. There shall be a pediatric on-call panel that allows for telephone consultation. This pediatrician shall be board certified or board eligible.
 - 2. A written plan shall exist whereby other pediatric specialists may be consulted and available in at least the following specialties: surgery, orthopedics, anesthesia, and neurosurgery. This requirement may be met by a written agreement with a PCCC.
 - 3. A plan shall exist whereby a second emergency physician or pediatrician will be promptly available to serve as back-up for the ED in critical situations.
- D. Mid-level Practitioner-Qualifications/Education*
 - 1. Physician Assistant (PA) or Nurse Practitioner (NP) licensed by the State of California.
 - 2. PA or NP working in the ED shall complete four hours of pediatric education yearly.
 - 3. Complete pediatric competency evaluations annually that are age specific and include neonates, infants, children/minors, adolescents, and children/minors with special healthcare needs.

*If applicable to the hospital

IV. EDAP POLICIES, PROCEDURES, AND PROTOCOLS

- A. Establish protocols and procedures for pediatric emergency patients to include, but not be limited to:
 - 1. Triage and initial evaluation
 - 2. Pediatric assessment
 - 3. Patient safety
 - 4. Suspected child/minor abuse and neglect
 - 5. Transfers
 - 6. Consents
 - 7. Sedation/analgesia
 - 8. Radiation dosage protocol
 - 9. Mental health emergencies
 - 10. Pain assessment and treatment
 - 11. Do-Not-Resuscitate (DNR)/Advanced Health Care Directives
 - 12. Death to include SIDS and the care of the grieving family
 - 13. Aeromedical transport to include landing procedure
 - 14. Daily verification of proper location and functioning of equipment and supplies of the pediatric code cart
 - 15. Immunizations
 - 16. Child/minor abandonment to include a recent (within 72 hours) postpartum woman without evidence of a newborn

17. Family centered care
18. Disaster preparedness written plan that addresses the following pediatric issues:
 - a. A plan to minimize parent-child/minor separation and improved methods for reuniting separated children/minors with their families
 - b. A plan that addresses pediatric surge capacity for both injured and non-injured children/minors
 - c. A plan that includes access to specific medical and mental health therapies, as well as social services, for children/minors in the event of a disaster
 - d. A plan which ensures that disaster drills include a pediatric mass casualty incident once every two years
 - e. Decontamination
- B. Establish a written interfacility consult and transfer agreement with a Pediatric Critical Care Center (PCCC) to facilitate transfers of critically ill pediatric patients to a Pediatric Trauma Center (PTC) or Medical Pediatric Critical Care Center (MPCCC). The consult service shall be available 24 hours a day for telephone consultation
- C. Establish a written interfacility consult and transfer agreement with a California Children Services (CCS) approved Level II or Level III Neonatal Intensive Care Unit (NICU)

V. EDAP PROCESS IMPROVEMENT (PI)

- A. A pediatric PI program shall be developed and monitored by the EDAP Medical Director and Pediatric Liaison Nurse with input from the designated Pediatric Consultant as needed.
- B. The program should include an interface with prehospital care, ED, trauma, pediatric critical care, pediatric inpatient, and hospital wide PI activities
- C. A mechanism shall be established to easily identify pediatric (children/minors less than 15 years of age) visits to the ED
- D. The pediatric PI program should include identification of the indicators, methods to collect data, results, conclusions, recognition of improvement, actions taken, assessment of effectiveness of actions, and communication process for participants
- E. Periodic review of aggregate data of pediatric emergency visits
- F. The pediatric PI program should include review of the following pediatric patients seen in the ED:
 1. Deaths
 2. Cardiopulmonary and/or respiratory arrests, including all pediatric intubations
 3. Suspected child/minor abuse or neglect
 4. Transfers to and/or from another facility

5. Trauma admissions from the ED
 6. Operating room admissions from the ED
 7. Admissions from the ED to any inpatient units where pediatric patients may be admitted
 8. Select repeated visits to the ED
 9. Patient safety including adverse events involving medication delivery
- G. A mechanism to document and monitor pediatric education of EDAP staff shall be established
- H. Pediatric clinical competency evaluations should be developed for all licensed ED staff. Competencies should be age specific and include neonates, infants, children/minors, adolescents, and children/minors with special healthcare needs. Competencies may include, but not be limited to:
1. Airway Management
 2. Burn care
 3. Critical care monitoring
 4. Medication delivery and device/equipment safety
 5. Pain assessment and treatment
 6. Trauma care
 7. Vascular access

VI. EDAP SUPPORT SERVICES

- A. Registered Respiratory Therapy
1. At least one registered respiratory therapist shall be in the hospital 24 hours a day
 2. PALS current
 3. Complete pediatric competency evaluations annually that are age specific and include neonates, infants, children/minors, and adolescents
- B. Radiology
1. Radiologist on call and promptly available 24 hours per day
 2. Radiology technician in house 24 hours per day
 3. CT scan technician on call and promptly available
- C. Laboratory
1. Technician in house 24 hours per day
 2. Clinical Laboratory capabilities in house:
 - a. Chemistry
 - b. Hematology
 - c. Blood Bank
 - d. Arterial blood gas
 - e. Microbiology
 - f. Routine toxicology and drug levels

VII. EDAP EQUIPMENT, SUPPLIES, AND MEDICATIONS

Pediatric equipment, supplies, and medications shall be easily accessible, labeled, and logically organized. EDAP staff will be appropriately educated as to the locations of all items.

Each EDAP will have a method of daily verification of proper location and function of equipment and supplies. It is highly recommended that each EDAP have a mobile crash cart. Trays and other items may be housed in other departments, (for example, Newborn Nursery or Central Supply) as long as the items are immediately accessible to the ED staff.

The following are requirements for equipment, supplies, and medications for an EDAP:

GENERAL EQUIPMENT

- Foley catheters (8-22fr)
- IV blood/fluid warmer
- Length based tape for estimating weights for purposes of determining pediatric medication dosages
- Meconium aspirator
- OB kit
- Pain assessment tools appropriate for age
- Posted or readily available pediatric drug dosage reference material calculated on a dose per kilogram basis
- Restraint device
- Scale capable of providing weight in kilograms
- Warming device

MONITORING EQUIPMENT

- Blood pressure cuffs (infant, child, adult, and thigh)
- Doppler
- Electrocardiogram (ECG) monitor/defibrillator with pediatric and adult paddles
- End tidal Carbon Dioxide (CO₂) monitor or detector (adult and pediatric sizes)
- Hypothermia thermometer
- Pulse Oximeter

RESPIRATORY EQUIPMENT

- Bag-valve mask device, self inflating (pediatric size: 450-900ml and adult size: 1000-2000ml)
- Bag-valve, with clear masks (neonate, infant, child, and adult sizes)
- Endotracheal tubes (cuffed and/or uncuffed: 2.5-5.5 and cuffed: 6.0-9.0)
- Laryngoscope (appropriate size range of curved and straight)
- Magill forceps (pediatric and adult)
- Nasal cannulae (infant, child, and adult)
- Nasopharyngeal airways (infant, child, and adult)
- Nasogastric tubes (including 5 French (Fr) and 8 Fr feeding tubes)
- Oral airways
- Clear oxygen masks (standard and reservoir) for infant, child/minor, and adult
- Stylets for endotracheal tubes (pediatric and adult)
- Suction catheters (sizes 6 Fr-12 Fr)
- Tracheostomy tubes
- Yankauer suction tips

VASCULAR ACCESS EQUIPMENT

- Arm boards (infant, child/minor, and adult)
- Infusion device to regulate rate and volume
- Intraosseous needles
- IV administration sets with calibrated chambers
- IV catheters (14G-24G)
- IV solutions (D5.2NS, D5.45NS, D5NS, D10W, and NS)
- Stopcocks (3-way)
- Umbilical vein catheters

FRACTURE MANAGEMENT DEVICES

- Pediatric spinal immobilization devices
- Pediatric femur splint
- Spine board

SPECIALIZED TRAYS OR KITS

- Cricothyrotomy tray to include equipment to perform needle cricothyrotomy
- Difficult airway supplies/kit
- Newborn delivery kit (kit with equipment for initial resuscitation of a newborn: umbilical clamp, scissors, bulb syringe, and towel)
- Pediatric lumbar puncture tray
- Pediatric tracheostomy tray
- Thoracostomy tray
- Chest tubes (sizes 10 Fr-28 Fr)
- Venous cutdown tray

PEDIATRIC SPECIFIC RESUSCITATION MEDICATIONS

- Resuscitation *medications per the American Heart Associations (AHA) PALS guideline.

***NOTE: It is suggested that these drugs be immediately available in the resuscitation room and not locked in a computerized system.**

VIII. INITIAL EDAP DESIGNATION PROCESS:

A hospital must follow these instructions to initiate the process to obtain recognition as an Emergency Department Approved for Pediatrics (EDAP).

A. Application Instructions

1. A hospital choosing to participate in the voluntary facility recognition process submits a letter of intent to the Solano County EMS Agency.
2. Within two weeks, Solano County EMS personnel will review the letter of intent and send a letter of receipt outlining next steps.
3. A formal application is required. The EDAP Application Attachment A and EDAP Application Attachment B can be found at the Solano County EMS website under the forms tab. The application fee should be submitted with the application.
4. Solano County EMS personnel will review the application within 90 days of receipt.
5. Within 90 days following receipt of the application and supporting documents, the hospital will be informed as to the status of the application. If all documentation is in order, a site visit will be scheduled. A letter to the hospital will be sent providing the date of the visit four to eight weeks before the site survey is conducted. Evaluation tools used by the site survey team will be included as an attachment to the letter.

B. Site Survey Process

1. Site survey team arrives at hospital (see Attachment E section I)
2. Opening conference (see Attachment E section II)
3. Emergency Department, Pediatric unit*, and Neonatal Intensive Care Unit tours (see Attachment E section III)
4. Review of documents and manuals (see Attachment E section IV)
5. Exit meeting (see Attachment E section V)

*If applicable to the hospital

C. Site Survey Team

1. The site survey team will be comprised, at a minimum, of a physician and nurse team along with a representative(s) from the Solano EMS agency. All team members will be given information regarding the site survey responsibilities, expectations, process, and assessment.

E. Following the Site Survey

1. Within six weeks following the site visit, the hospital will receive the results of the survey. Hospitals meeting all requirements will receive a letter from the Solano EMS agency formally recognizing the hospital as EDAP certified.
2. Hospitals that do not meet the requirements will receive a letter from the Solano EMS agency outlining the areas of non-compliance. Any outstanding unmet criteria identified during the site visit must be addressed within six weeks. If deemed necessary, another site survey may be scheduled.
3. Withdrawal of recognition status may occur at any time should a hospital fail to meet any of the requirements. In this situation, the hospital will notify Solano County EMS at least 60 days prior to withdrawal and identify how area prehospital provider agencies, area hospitals, and Solano County EMS Agency will be notified.

IX. EDAP Renewal Process

- A. Renewal of EDAP designation will occur annually, with site visits scheduled as necessary. Hospitals must submit the renewal application and fees along with supporting documentation.
- B. A letter will be sent to the hospitals notifying them of when Solano County EMS is accepting applications for renewal. Applications should be submitted by the due date announced by the agency.

X. Destination Protocols for Ambulances:

In all cases, the health and well-being of the pediatric patient is the overriding consideration in determining patient destination. Factors to be considered include severity and stability of the pediatric patient's illness or injury; current status of the pediatric receiving facility; anticipated transport time; and request by the patient, family guardian, or physician.

For purposes of this policy, the closest receiving center is the most accessible EDAP.

A. Critically Injured Child/Minor:

Paramedics will transport critically injured children/minors to the closest Pediatric Trauma Center.

B. Critically Ill Child/Minor:

Paramedics will transport critically ill children/minors to the closest EDAP.

C. Non-Critical Child/Minor:

All children/minors entering the EMS system who require ambulance transport, and are critically ill or injured, will be transported to the closest EDAP. However, for children/minors who are NOT critically ill or injured, requests for transport to a Non-EDAP (or more distant EDAP) may be honored. Such requests will require signature by the patient's legal guardian. Paramedics will ensure documentation of this request within the patient's PCR and submit the Non-EDAP Acknowledgement Form to the EMS.

If a parent or guardian is not present, the minor will be transported to the closest EDAP.

D. Non-EDAP Transport Acknowledgement Form:

1. Paramedics will inform the parent or person legally responsible for the child that a form requesting transportation to an emergency department that has not been designated as an EDAP must be completed.
2. Original copies will be submitted by the transporting agency by the 15th of every month.

***Note:** See Attachment G for a copy of the Non-EDAP Transport Acknowledgment Form.

XI. Solano EMS Data Sharing and Meeting Participation Requirements

Attendance of PI/EDAP and Trauma Advisory Committee (TAC) meetings are mandatory; and attendance of STEMI meetings is highly recommended.

Data sharing is an important aspect of process improvement and efficiency of prehospital care. Representatives from the EDAP designated hospitals will present the following information at the EMS Quarterly meetings:

1. Data on system performance for each EDAP hospital. The content of the presentation should be focused on identifying opportunities for improvement and to recognize outstanding performance given by prehospital personnel. The presentation should include patient interfacility transfers, and patient outcome.
2. Cases that involve the three required pediatric process improvement monitors:
 - a. Pediatric Deaths
 - b. Pediatric Interfacility Transfers
 - c. Suspected child/minor abuse/neglect
3. Cases that did not follow Policy 7300

ALS field providers must comply with all policies and protocols used to care for pediatric patients. ALS PI leaders are required to submit pediatric-related data including:

- Monthly list of all pediatric patients treated under pediatric protocols by the 15th day of each month. (i.e., March data due April 15th)

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Solano County
Department of Health and Social Services
Emergency Medical Services Agency
Emergency Department Approved for Pediatrics

Application

An annual application/designation fee of \$5,000 is required.
Please make check payable to: Solano County Emergency Medical Services Agency.

General Information

Name of Hospital: _____

Address: _____

Phone: _____

California Department of Public Health License (CDPH)

*exp. date: _____

The Joint Commission (THC) Accreditation

*exp date: _____

Submit copy of certificate(s)

Administration/Coordination

Chief Executive Officer: _____ Phone: _____

EDAP Medical Director: _____ Phone: _____

Designated Pediatric Consultant: _____ Phone: _____

Pediatric Liaison Nurse: _____ Phone: _____

ED Director/Manager: _____ Phone: _____

EDAP APPLICATION

Personnel- Physicians

- Twenty-four hour emergency department coverage shall be provided or directly supervised by physicians functioning as emergency physicians or pediatricians experienced in emergency care on a full time basis (96 hour or more per month in an emergency department). This includes senior residents practicing at their respective hospital only.
- At least 75% of the emergency department coverage shall be provided by physicians Board certified or eligible in emergency medicine or pediatrics.
- Emergency department physicians (other than the EDAP Medical Director) who are not Board certified or eligible shall be a current APLS or PALS provider.
- **Have available on the day of survey – proof of Board Certification certificate(s) and AHA PALS card or APLS card.**

NAME	* DATE BOARD ELIGIBLE	*BOARD CERT EXP. DATE.	HOURS WORKED PER MONTH	*PALS/APLS EXP. DATE
EDAP Medical Director				

EDAP APPLICATION

[illegible]

EDAP APPLICATION

PERSONNEL- NURSES

- At least 75% of the total RN staff and at least one RN per shift in the emergency department shall be current PALS or APLS provider.
- At least one RN per shift shall have completed ENPC or a two day pediatric emergency nursing course.
- NOTE: It is highly recommended that all nurses regularly assigned to the emergency department meet the above requirements.
- All nurses assigned to the emergency department shall attend, at a minimum; four hours of pediatric BRN approved education every year.
- **Submit the appropriate documentation to verify AHA PALS status and education (copy of AHA PALS cards, copy of the two day course certificates and additional Peds CE certificates).**

NAME	*ENPC/PALS/APLS EXP. DATE	*DATE OF LAST TWO DAY PEDS COURSE	*PEDS BRN HOURS IN LAST TWO YEARS
Pediatric Liaison Nurse			

EDAP APPLICATION

[illegible]

EDAP APPLICATION

PEDIATRIC ON CALL PANEL

- Establishment of a pediatric on call panel that allows for telephone consultation and a promptly available pediatrician to the emergency department twenty-four hours per day. Physicians listed on the On Call Panel shall be board certified or eligible.
- **Have available on the day of survey – proof of Board Certification certificate(s).**

NAME	*DATE BOARD ELIGIBLE	*DATE BOARD CERT EXP
Designated Pediatric Consultant		

EDAP APPLICATION

ED MID-LEVEL PROVIDERS AND RESPIRATORY THERAPY

- **Submit the appropriate documentation to verify, at minimum, four hours of pediatric education every year.**

[illegible]

EDAP APPLICATION

[illegible]

**Solano County
Department of Health and Social Services
Emergency Medical Services Agency
Emergency Department Approved for Pediatrics**

ADMINISTRATION

Submit curriculum vitae for the following personnel:

- EDAP Medical Director
- Designated Pediatric Consultant
- Pediatric Liaison Nurse

PERSONNEL

Submit the last three months of the following staffing schedules:

- Emergency Department Physicians
- Emergency Department Nurses
- Pediatric On-Call Panel Schedule

POLICIES, PROCEDURES, AND PROTOCOLS

Submit the following:

- Triage and initial evaluation of the pediatric patient
- Pediatric assessment
- Pediatric patient safety
- Suspected child abuse and neglect
- Transfers
- Consents
- Conscious sedation of the pediatric patient
- Analgesia
- Radiation dosage protocol
- Mental Health emergencies
- Pain assessment and treatment
- Do-not-resuscitate (DNR)/Advanced Health Care Directive
- Death to include Sudden Infant Death Syndrome (SIDS) and the care of the grieving family
- Aeomedical transport to include landing procedure*
- Daily verification of proper location and functioning of pediatric specific equipment and supplies
- Immunizations
- Child abandonment to include a recent (within 72 hours) postpartum woman without evidence of a newborn
- Family patient care

- Disaster Preparedness plan that addresses the following pediatric issues:
 1. A plan to minimize parent-child separation and improved methods for reuniting separated children with their families
 2. A plan that addresses pediatric surge capacity for both injured and non-injured children
 3. A plan that includes access to specific medical and mental health therapies, as well as social services, for children in the event of a disaster
 4. A plan which ensures that disaster drills include a pediatric mass casualty incident once every two years.
 5. Decontamination
- Interfacility consult and transfer agreement with a Pediatric Critical Care Center (PCCC) to facilitate transfers of critically ill and injured patients (PTC). The consult shall be available twenty four hours a day for telephone consultation.
- Interfacility consult and transfer agreement with a California Children Services (CCS) approved Level II or III Neonatal Intensive Care Unit (NICU)

QUALITY IMPROVEMENT (QI)

Submit the **EDAP QI Plan** which includes the following:

- Goal/Mission Statement
- Authority and responsibilities of the EDAP Medical Director, PdLN, and Pediatric Consultant
- Interface with prehospital care, emergency department, trauma*, pediatric critical care*, pediatric inpatient*, and hospital-wide QI activities
- Mechanism of the identification of pediatric (up to 15 years of age and under) visits to the emergency department
- Identification of the indicators, methods to collect data, results and conclusions, recognition of improvement, action(s) taken, assessment of effectiveness of above actions and communications process for participants
- QI reports for the following pediatric patients seen in the emergency department:
 1. Deaths
 2. Cardiopulmonary and/or respiratory arrests, including all pediatric intubations
 3. Suspected child abuse or neglect
 4. Transfers to and/or from another facility
 5. Trauma admissions from the ED
 6. Operating room admissions from the ED
 7. Admissions from the ED to an adult ward or ICU*
 8. Selected return visits to the ED
 9. Patient safety including adverse events involving medication
 10. CCT or Emergency ALS Pediatric transports
- Documentation and monitoring of pediatric education to ED EDAP staff

- Pediatric clinical competency evaluations for licensed ED EDAP staff.
Competencies should be age specific and include neonates, infants, children, adolescents and children with special healthcare needs.

1. Airway Management
2. Burn care
3. Critical care monitoring
4. Medication delivery, and device/equipment safety
5. Pain assessment and treatment
6. Trauma care
7. Vascular access

*If applicable to your hospital

EDAP STANDARDS

INTRODUCTION:

In order to address the quality of care provided to pediatric patients in Solano County, the EMS Agency has adopted an Emergency Department Approved for Pediatrics (EDAP) process. Approximately 30 million children are seen each year in our nation's emergency departments (ED's). Most of these children (90%) will arrive by private car and 10% will arrive by ambulance. Each ED must be prepared with appropriate staff, equipment, supplies, and procedures to provide quality care regardless of the patient's age or presenting complaint.

These processes will ensure that all trauma centers and designated emergency departments that receive pediatric patients through the 9-1-1 system have the appropriately trained staff and the necessary equipment to provide quality care for those pediatric patients.

The Solano County EDAP standards meet or exceed the standards established by the Emergency Medical Services for Children (EMSC) administration, personnel, and policy guidelines for the care of pediatric patients in the emergency department set forth by the California Emergency Services Authority in 1995. These guidelines are specifically designed to provide ED's with the necessary resources for the care of children in the emergency setting.

Without a pediatric critical care center or Children's hospital, Solano County has few to little resources to offer pediatric disaster victims. The EDAP process by design will ensure rigorous standards are set, adhered to, regulated and sustained in order to address factors such as pediatric emergency readiness, pediatric disaster preparedness, and acute care and appropriate transfer of the pediatric patient.

In addition to being a resource document explaining and detailing the EDAP process the application at the end of this manual will serve as request and guideline for designation by the Solano County EMS Agency as an EDAP.

DEFINITIONS:

APLS: American Academy of Pediatrics – American College of Emergency Physicians Advanced Pediatric Life Support Course

Board Certified: Completed an approved educational training program and an evaluation process including an examination designed to assess the knowledge, skills, and experience necessary to provide quality care in that specialty.

Board Prepared: Successful completion of a Board approved emergency medicine or pediatric residency training program and demonstrate active progression in the certifying process.

Children With Special Health Care Needs (CSHCN): Children who have any type of condition that may affect normal growth and development. This may include physical disability, developmental or learning disability, technologic dependency, and/or chronic illness. The developmental age of CSHCN may be younger than the chronological age. The developmental age is used when working with this population.

Community Pediatric Center: A hospital with basic emergency services, staffed with a physician 24/7, but which may have limited inpatient services. The CPC is a general community hospital that has an adult inpatient specialty care and has no dedicated inpatient pediatric services; however, diagnostic, operative, and therapeutic services and equipment, and selected physician specialists are available for consultation.

Continuous Quality Improvement (CQI): Methods of evaluation that are comprised of structure, process, and outcome evaluations which focus on improvement efforts to identify root causes of problems, intervene to reduce or eliminate these causes, and take steps to correct the process. CQI recognizes excellence in performance and delivery of care and can be system wide or programmatic.

Emergency Department Approved for Pediatrics (EDAP): A licensed basic emergency department that is approved by the County of Solano to receive pediatric patients from the 9-1-1 system. These emergency departments provide care to pediatric patients by meeting specific requirements for professional staff, quality improvement, education, support services, equipment, supplies, medications, and established policies, procedures, and protocols.

Emergency Medical Services Authority: “EMSA” or “Authority” is the department in California responsible for the coordination and integration of all EMS related state activities.

Emergency Medical Services for Children: Resources specific for the care of children within EMS systems with the following components: prevention, prehospital, emergency department (ED), in-patient, and rehabilitation services.

ENPC: Emergency Nurses Association – Emergency Nursing Pediatric Course

Medical Pediatric Critical Care Center (MPCCC): A licensed acute care hospital that is approved by a local EMS agency to receive critically ill non-trauma pediatric patients from the 9-1-1 system.

MID-LEVEL PRACTITIONER: Either a physician assistant or nurse practitioner licensed in the State of California.

PALS: American Heart Association Pediatric Advanced Life Support Course

Pediatric Critical Care Center (PCCC): A licensed acute care hospital that is approved by a local EMS agency to receive patients from the 9-1-1 system. In addition, this center provides tertiary-level pediatric care services and serves as a referral center for critically ill and injured pediatric patients.

Pediatric Patient: Any persons up to their fifteenth birthday.

Pediatric Receiving Center: A licensed general acute care hospital with, at a minimum, a permit for basic emergency services that has been formally designated by the LEMSA for its role in EMS system.

Pediatric Resuscitation Area: A designated area within the emergency department within a PRC where a pediatric resuscitation may be initiated.

Promptly available: Being in the emergency department within a period of time that is medically prudent and appropriate to the patient's clinical condition; and further, that the interval between the arrival of the patient to the emergency department and the arrival of the respondent should not have a measurably harmful effect on the course of patient management or outcome.

PTC: Pediatric Trauma Center

Qualified specialist: A physician licensed in the State of California who has : 1) taken special postgraduate medical training, or has met other specified requirements; and 2) active progression towards board certification in the corresponding specialty for those specialties that have board certification and are recognized by the American Board of Medical Specialties.

Senior resident: A physician licensed in the State of California who has completed at least two years of the residency under consideration and has the

capability of initiating treatment when the clinical situation demands, and who is in training as a member of the residency program at the designated hospital.

Trauma Center: A licensed hospital which has been designated as a Level I, II, III, or IV trauma Center and/or Level I or II Pediatric Trauma Center by the local EMS agency.

Triage: Measures or methods of assessing the severity of the patient's illness or injury using anatomic, physiologic considerations, and/or mechanism of injury to establish a prioritization of care.

Site Survey Process:

I. Recommendations for Hospital Staff

- A. A hospital staff member should meet the survey team in the main lobby of the hospital at the time specified and escort them to the designated room for the opening conference.
- B. Please reserve a conference room that will be available for the duration of the survey and is large enough to accommodate the survey team and hospital staff comfortably. This room will be utilized for the opening and exit conferences as well as the review of requested documents and manuals. The room should be located away from patient care areas.
- C. Hospital staff should provide a guided tour of the following areas:
Emergency Department, Pediatric Unit, Neonatal Intensive Care Unit, and any inpatient units where pediatric patients may be admitted.

II. Opening Conference – Approximately 30 minutes

- A. Hospital administration should choose personnel to attend the opening conference. The following individuals (or their designees) are recommended (other individuals are welcomed to attend):
 - Chief Administrative Officer/Executive Officer, or designee
 - Chief of Pediatrics or Designated Pediatric Consultant
 - Administrator of Pediatric Services, if applicable
 - Nursing Director or Nurse Manager of the Pediatric Units
 - Emergency Department Medical Director and/or Pediatric ED Medical Director
 - Emergency Department Medical Director and/or Pediatric ED Medical Director
 - Emergency Department Nurse Manager and/or Pediatric ED Nurse Manager
 - Administrator of Emergency Services
 - EDAP Medical Director
 - Pediatric Liaison Nurse
 - Hospital Quality Improvement Department Director or designee
 - Mid-Level Practitioner, e.g., Nurse Practitioner or Physician Assistant for those facilities that utilize mid-level practitioners in their ED

Note: Personnel representing the EMS Agency will be in attendance.

- B. The opening conference begins with introductions of the survey team and hospital representatives. The site survey team leader will provide a brief Emergency Medical Services for Children (EMSC) overview and outline the site survey agenda.

- C. Hospital staff should then be prepared to conduct a brief formal presentation on the following:
1. Provide a presentation using the “Strength, Weaknesses, Opportunities, Threats” (SWOT) format. Outline strengths, weaknesses, opportunities, and threats as related to pediatric emergency care capabilities, services, and resources associated with being an EDAP.
 2. Present the following demographic information:
 - Age range of pediatric patient population;
 - Annual number of emergency department visits (including a pediatric breakdown);
 - Top five ED pediatric diagnoses;
 - Annual number of pediatric inpatient admissions* from the ED and top five admitting diagnoses*.
 3. Provide an overview of the ED QI process and activities. The EDAP Medical Director and Pediatric Liaison Nurse should review the following:
 - Discuss how pediatrics is integrated into the QI process;
 - Discuss reporting mechanisms/loop closure mechanisms;
 - Identify how the Pediatric Liaison Nurse is supported in their roles.
 4. Discuss the interfacility transfer process, including:
 - The volume/types of pediatric patients transferred into and out of the facility;
 - Transport team access, response, composition and capabilities.
 5. Outline how pediatric disaster preparedness and pediatric surge capabilities are being addressed in disaster planning.

III. Emergency Department, Pediatric Unit, NICU, and Inpatient Units (admitting pediatric patients) Tours

- A. After completion of the opening conference, the ED/Pediatric/NICU and pediatric inpatient unit tour will begin. The survey team will encourage dialogue during the tour and may utilize tour time to conduct informal conversations with clinical staff to assess level of awareness and knowledge regarding pediatric care and policy and procedures.
- B. Tour will focus on patient flow from patient arrival through disposition and should include identification of pediatric designated area/beds (if applicable).
- C. Designated hospital staff will provide a guided tour for site surveyors that shall include the following area:
 - Ambulance bay
 - Helicopter landing area
 - ED area including neonatal and pediatric beds/area if designated

- Location and access to on-call schedule(s) for specialty services and resource staff
- EMS Communication center or equipment
- Pediatric equipment/supplies/medications areas. Compliance with equipment/supplies criteria will be reviewed during the tour. Please ensure that staff is available to assist with this.
- Location of Radiology/CAT scan in proximity to the ED
- Disaster supply area(s), decontamination shower area/capabilities, and pediatric surge areas.

IV. Review of Documents and Manuals (Approximately 60-75 minutes)

Please assure that the following documents and manuals are available in the conference room for the site survey team to review after the tour has been completed:

1. Any items/documents/clarification requested in your EDAP application must be provided.
2. Emergency department policy and procedure manuals
3. ED Nursing and physician staff meeting minutes
4. Nursing and physician staff CE files
5. Hospital Emergency Operations Plan (EOP)/Disaster Plan. In addition, a completed EMSC Hospital Pediatric Preparedness Checklist should be available for review.
6. Emergency department QI documentation/manuals with monitor tools, follow-up/loop closure documentation and multidisciplinary QI meeting minutes. QI documentation related to the following must be available: pediatric deaths, interfacility transports, child abuse/neglect, critically ill or injured children in need of stabilization (e.g., respiratory failure, sepsis, shock, altered level of consciousness, cardiopulmonary failure), and pediatric strategic priorities of the institution.

NOTE: Please be prepared to have the EDAP Medical Director and Pediatric Liaison Nurse walk through the QI review process for each of the three EDAP required QI monitors (see below). This review should include one patient medical record for each of these three monitors to demonstrate the review process for each monitor (i.e., have a medical record for a non-critically ill/injured child and walk through the review process for that pediatric patient). Be sure to have available any related QI tools/parameters for each monitor during this review.

- Pediatric deaths
- Pediatric interfacility transfers
- Suspected child abuse/neglect

7. Documentation related to current or planned pediatric mock code disaster drills.
8. Transfer log and transfer/transport QI documentation.
9. Any other documents requested by the site survey team at the time of survey.

V. Exit Meeting (Approximately 20-30 minutes)

- A. The exit meeting is the culmination of the site survey visit. In order to obtain maximum benefit of the exit meeting, it is recommended that the same hospital personnel who attend the opening conference be present for the exit meeting.
- B. The exit conference will be tape recorded to assure that discussions and recommendations for the post-survey written report are captured accurately. The hospital is also encouraged to tape record for future reference.
- C. The purpose of the exit meeting is to provide an opportunity for the site survey team to obtain clarification on any remaining questions identified during the tour/quality improvement review and to provide the institution with an opportunity to contribute any additional information as necessary.
- D. The exit meeting and survey process should be viewed as a means of identifying opportunities for improvement in pediatric emergency care. The Solano County EMS agency is committed to assisting hospitals with their efforts to improve their emergency medical services and critical care capabilities for children.

Solano County Public Health
Emergency Medical Services
Emergency Department Approved for Pediatrics

SITE VISIT

HOSPITAL:

DATE:

TIME:

OPENING CONFERENCE

- Site Survey team leader, Nancy McGrath, will provide a brief Emergency Medical Services for Children (EMSC) overview and outline the site survey agenda.
- Hospital representatives will conduct a brief presentation as outlined in Attachment E.

EQUIPMENT, SUPPLIES, AND MEDICATION

- The team will tour the emergency department/NICU/Pediatric* areas and any inpatient units where pediatric patients may be admitted
- Following the tour, the site survey team will perform an inventory inspection of the required equipment, supplies, and medications.

PERSONNEL REQUIREMENTS

- Provide evidence which shows the certifications for the ED physicians, Pediatric physicians, mid-level practitioners, and respiratory therapy.

NURSES QUALIFICATIONS / EDUCATIONAL REQUIREMENTS

- Provide evidence (certifications) which shows the nursing qualifications and how pediatric continuing education attended by nursing staff is monitored.

MEDICAL RECORD REVIEW

Please have available for review three recent (within 90 days) pediatric emergency department charts for each of the following categories:

- Acute Respiratory Distress
- Fever less than three months of age
- Child Abuse

- Unplanned return visits
- Acute dehydration
- Seizure
- Transfer to a PICU

QUALITY IMPROVEMENT

Review the Emergency Department Pediatrics QI Plan which includes the following:

- Goal/Mission statement
- Authority and responsibilities of the EDAP Medical Director, Pediatric Nurse Liaison, and Pediatric Consultant
- Interface with prehospital care, emergency department, trauma, pediatric critical care, and hospital-wide QI activities
- Identification of the indicators, methods to collect data, results and conclusions, recognition of improvement, action(s) taken, assessment of effectiveness of above actions and communication process for participants
- QI reports for the following pediatric patients seen in the emergency department:
 1. Deaths
 2. Cardiopulmonary and/or respiratory arrests, including all pediatric intubations
 3. Suspected child abuse or neglect
 4. Transfers to and/or from another facility
 5. Admissions from the ED to any inpatient units where pediatric patients may be admitted
 6. Select repeated visits to the ED
 7. Pediatric transports within the 9-1-1 system



Non-EDAP Transport Acknowledgement Form

I acknowledge I am requesting transportation to an emergency department that has not been designated as an Emergency Department Approved for Pediatrics (EDAP).

Minor's Name (Print)

Parent/Guardian Name (Print)

Date

Parent/Guardian Signature

Run Number: _____

PCR Number: _____